

D545

Healthcare Administration Evolution, Systems, and Leadership

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 300-bed community hospital with a new CEO and a strong nursing school partnership wants to keep its clinics open in the evening to improve access. Doing so will add staffing cost. According to the iron triangle, what must leaders consider?

- A. Improving access affects cost and possibly quality, so the tradeoff must be planned explicitly.
- B. Access can improve with no effect on cost or quality.
- C. The iron triangle is about nurse staffing ratios.
- D. Only cost matters to the decision.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A 67-year-old retired teacher, a patient at a clinic known for its diabetes program, needs coverage for a hospital stay. Her income is moderate and she has no disability. Which public program is her primary coverage?

- A. Medicaid
- B. Medicare Part D
- C. Medicare Part A
- D. No public coverage until age 70

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3. [TRANSFER — reversed stem / different variable]

Which program is funded jointly by the federal and state governments and covers eligible low-income people?

- A. Medicare
- B. Medicare Advantage
- C. TRICARE
- D. Medicaid

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4.

A primary care group of 20 physicians, recently named a top workplace, is paid a fixed \$50 per member per month for 10,000 patients, no matter how many visits they make. What incentive does this create?

- A. More visits bring more revenue.
- B. Keep patients healthy and manage use, with a risk of underservice.
- C. No financial incentive at all.
- D. Keep each surgical episode under a target price.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5. [TRANSFER — reversed stem / different variable]

Under fee-for-service, what is a provider's financial incentive?

- A. Provide fewer services.
- B. Keep total episode cost under a target.
- C. Maximize quality scores.
- D. Provide more services.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

A state is considering a rule requiring hospitals to post prices online. The hospital association, a large insurer, a patient advocacy group, and the state health department all testify at a hearing held in the new capitol annex. Which stakeholder is the regulator?

- A. The large insurer
- B. The patient advocacy group
- C. The state health department
- D. The hospital association

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

A nursing home administrator, who holds an MBA and runs marathons, faces low morale after a merger. She sets out a clear vision, involves staff in redesigning workflows, and coaches individuals toward growth. Which leadership style is this?

- A. Laissez-faire
- B. Transactional
- C. Transformational
- D. Autocratic

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A health system, which recently opened a cancer center, gives every patient the same printed discharge instructions in English. Readmissions are much higher among patients with limited English. Which approach reflects health equity?

- A. Keep identical instructions, since treating everyone the same is fair.
- B. Provide interpreters, translated instructions, and teach-back for patients with limited English.
- C. Translate materials only for the new cancer center.
- D. Stop tracking readmissions by language.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

Which is a social determinant of health rather than a clinical factor?

- A. Blood pressure
- B. Access to reliable transportation
- C. Medication dosage
- D. A genetic mutation

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A 12-hospital health system, famous for its heart program, uses an EHR that cannot share records with outside clinics, so patients repeat tests. Which capability addresses this?

- A. Interoperability through standards such as HL7 FHIR and a health information exchange.
- B. Adding more internal EHR modules.
- C. Offering telehealth visits.
- D. Buying a new EHR, since any new system shares data automatically.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A rural hospital had operating revenue of \$80 million and operating expenses of \$78 million. It also received a one-time \$3 million donation for a new wing, named for a local family. What is its operating margin?

- A.** 97.5%
- B.** About 2.56%
- C.** 6.25%
- D.** 2.5%

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

12. [TRANSFER — reversed stem / different variable]

A clinic targets a 4% operating margin on \$25 million of operating revenue. What is the most it can spend in operating expenses?

- A.** \$24 million
- B.** About \$24.04 million
- C.** \$26 million
- D.** \$1 million

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	____	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A 300-bed community hospital ~~with a new CEO and a strong nursing school partnership~~ wants to keep its clinics open in the evening to improve access. Doing so will add staffing cost. According to the iron triangle, what must leaders consider?

Target word: tradeoff **Decides:** Better access raises cost.

A	Correct	The three sides move together.
B	Absolute language	The stem already names a cost increase.
C	Confusion set	It is about cost, quality, and access.
D	Dropped element	Drops quality and access.

2. Answer C Concept 2

A 67-year-old retired teacher, ~~a patient at a clinic known for its diabetes program~~, needs coverage for a hospital stay. Her income is moderate and she has no disability. Which public program is her primary coverage?

Target word: program **Decides:** Age 67.

A	Confusion set	Based on low income; her income is moderate.
B	Adjacent category	Part D covers prescription drugs, not hospital stays.
C	Correct	Hospital coverage for people 65 and older.
D	Wrong stem number	Medicare eligibility starts at 65.

3. Answer D Concept 2 · transfer

Which program is funded jointly by the federal and state governments and covers eligible low-income people?

Target word: reverse: description to program **Decides:** Joint funding and low income.

A	Confusion set	Federal, and mainly based on age.
B	Inverted operation	Private plans that deliver Medicare benefits.
C	Adjacent category	Covers military members and families.
D	Correct	A federal-state program based on income.

4. Answer B Concept 3

A primary care group of 20 physicians, ~~recently named a top workplace~~, is paid a fixed \$50 per member per month for 10,000 patients, no matter how many visits they make. What incentive does this create?

Target word: incentive **Decides:** Fixed payment per member (capitation).

A	Inverted operation	That is fee-for-service.
B	Correct	Revenue is fixed, so each extra service costs the group.
C	Absolute language	A fixed payment creates strong cost incentives.
D	Adjacent category	That is a bundled payment.

5. Answer D Concept 3 · transfer

Under fee-for-service, what is a provider's financial incentive?

Target word: reverse: model to incentive **Decides:** Payment per service.

A	Inverted operation	That is the capitation pressure.
B	Adjacent category	That is bundled payment.
C	Confusion set	That is value-based purchasing.
D	Correct	Each service is billed separately.

6. Answer C Concept 4

A state is considering a rule requiring hospitals to post prices online. The hospital association, a large insurer, a patient advocacy group, and the state health department all testify at a hearing ~~held in the new capitol annex~~. Which stakeholder is the regulator?

Target word: regulator **Decides:** A government agency sets and enforces rules.

A	Adjacent category	A payer.
B	Dropped element	Represents consumers; it has no enforcement power.
C	Correct	Government agencies regulate.
D	Confusion set	A trade group representing providers.

7. Answer C Concept 5

A nursing home administrator, ~~who holds an MBA and runs marathons~~, faces low morale after a merger. She sets out a clear vision, involves staff in redesigning workflows, and coaches individuals toward growth. Which leadership style is this?

Target word: style **Decides:** Vision, involvement, and development.

A	Adjacent category	Hands-off; she is actively coaching.
B	Confusion set	Relies on rewards for meeting targets.
C	Correct	Inspires and develops people toward a shared vision.
D	Inverted operation	Decides alone; she involves staff.

8. Answer B Concept 6

A health system, ~~which recently opened a cancer center~~, gives every patient the same printed discharge instructions in English. Readmissions are much higher among patients with limited English. Which approach reflects health equity?

Target word: equity response **Decides:** Outcome gap tied to language.

A	Confusion set	That is equality, not equity.
B	Correct	Tailors resources to need to close the outcome gap.
C	Camouflage	The gap is system-wide.
D	Inverted operation	Hides the gap.

9. Answer B Concept 6 · transfer

Which is a social determinant of health rather than a clinical factor?

Target word: reverse: classify **Decides:** Social and economic conditions.

A	Confusion set	A clinical measure.
B	Correct	A living condition that shapes health.
C	Confusion set	A treatment factor.
D	Adjacent category	A biological factor.

10. Answer A Concept 7

A 12-hospital health system, ~~famous for its heart program~~, uses an EHR that cannot share records with outside clinics, so patients repeat tests. Which capability addresses this?

Target word: capability **Decides:** Records cannot move between organizations.

A	Correct	Standards let records move between systems.
B	Adjacent category	Does not connect to outside clinics.
C	Confusion set	Improves access, not record sharing.
D	Absolute language	Sharing requires standards and connections.

11. Answer D Concept 8

A rural hospital had operating revenue of \$80 million and operating expenses of \$78 million. It also received a one-time \$3 million donation for a new wing, ~~named for a local family~~. What is its operating margin?

Target word: operating margin **Decides:** \$2M / \$80M; the donation is non-operating.

A	Inverted operation	Expenses as a share of revenue.
B	Wrong stem number	Divides by expenses instead of revenue.
C	Camouflage	Adds the non-operating donation: 5 / 80.
D	Correct	$(80 - 78) / 80 = 2.5\%$.

12. Answer A Concept 8 · transfer

A clinic targets a 4% operating margin on \$25 million of operating revenue. What is the most it can spend in operating expenses?

Target word: solve for expenses **Decides:** $25 \times (1 - 0.04)$.

A	Correct	\$25M – \$1M margin.
B	Adjacent category	$25 / 1.04$ treats the margin as a markup on cost.
C	Inverted operation	Adds the margin.
D	Confusion set	The margin in dollars.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owled.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	2	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	9	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	3	
Confusion set	Every option is real; the test is exact role-matching.	11	
Camouflage	Answers a stem fact that was never the question.	2	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	The iron triangle	Cost, quality, and access are linked; improving one usually affects the others.	Assuming one can improve with no effect on the others.	A plan to widen access or cut cost.
2	Medicare vs. Medicaid	Medicare is federal coverage mainly for people 65 and older (and some with disabilities or ESRD); Medicaid is joint federal-state coverage for eligible low-income people.	Mixing up the two programs.	Age, income, and disability facts about a patient.
3	Payment models and incentives	Fee-for-service rewards volume; capitation pays a fixed amount per member and rewards managing use; bundled payments cover an episode.	Thinking capitation rewards more visits.	“Per member per month” vs. “per service.”
4	Stakeholder roles	Providers deliver care, payers finance it, patients receive it, and regulators set and enforce rules.	Calling any powerful group a regulator.	Several groups testifying on a rule.
5	Leadership styles	Transformational leaders inspire with vision and develop people; transactional leaders trade rewards for performance.	Confusing involvement and coaching with transactional exchange.	Vision, involvement, and coaching in a change setting.
6	Health equity and social determinants	Equity means giving people what they need to reach equal outcomes; social and economic conditions shape health.	Treating identical treatment as equity.	Worse outcomes for one group under a one-size process.
7	Interoperability	Standards-based exchange (such as HL7 FHIR and health information exchanges) lets systems share records.	Assuming a new system shares data automatically.	Patients repeating tests because records do not move.
8	Operating margin	Operating margin = (operating revenue – operating expenses) / operating revenue; one-time gifts are non-operating.	Counting donations or dividing by expenses.	Operating figures plus a one-time donation.

Course scope (public description): Healthcare Administration Evolution, Systems, and Leadership provides an in- depth exploration of the U.S. healthcare system. The course covers the system’s evolution, key stakeholders, business principles, leadership roles, health equity issues, and the transformative influence of technology in healthcare administration. The learner will obtain tools necessary to drive positive change within the healthcare landscape through ample opportunity for strategic thinking and problem solving.